

Healthcare Regulatory Landscape Outlook



Horizon: **March - May 2026**

Carver Agents

Executive conditions through May 2026 are defined by **(1) accelerated post-market safety interventions for drugs and devices**, **(2) hard deadlines for health data / regulatory reporting infrastructure**, and **(3) payer and provider cost-controls paired with stepped-up fraud and competition enforcement**. In the EU/UK/AU, regulators are moving from guidance and voluntary reporting into **mandatory, systematized surveillance** (EUDAMED go-live obligations; Australia hospital device adverse-event reporting; repeated field safety notices and withdrawals). This materially raises compliance risk for manufacturers, importers, hospitals, and digital health vendors—especially those with weak UDI/master data, complaint handling, and CAPA execution.

In the US, the near-term operational burden is less about new federal statutes and more about **implementation and enforcement**: CMS prior-authorization process requirements are already in effect (decision timeframes, denial reasons, public metrics), states are tightening **utilization management and transparency**, and enforcement activity continues against **fraud, discrimination, and anti-competitive conduct**. The bottom line: organizations should treat March–May 2026 as a **readiness window**—to harden surveillance, data governance, and auditability—before mandatory reporting and database obligations create “no-excuses” compliance exposure.

Detailed Outlook Analysis

What is happening and what you need to do now to prepare.

1) Post-Market Safety Escalation (Devices + Digital Systems)

EU competent authorities are demonstrating willingness to use **police powers** to remove products and stop investigations when patient harm signals emerge. France's ANSM ordered sweeping restrictions on Medtronic's TouchCare sensor (marketing/import/distribution/advertising/use) and required withdrawal actions, while also terminating a related clinical investigation—an aggressive posture that signals **low tolerance for unresolved severe adverse-event profiles** ([ANSM](#)). In parallel, Germany's BfArM continues publishing urgent field safety notices across device categories (insulin pumps, endoscopic clips, PACS/reporting software, aesthetic lasers), underscoring that **software defects and use-conditions** are being treated as patient-safety issues requiring immediate corrective actions ([BfArM – Medtronic pumps](#); [BfArM – Spacelabs SENTINEL](#); [BfArM – DeepUnity PACSonWEB](#)).

Switzerland's Swissmedic FSCA notices on MAJ-210 biopsy valves (rubber fragment detachment with serious injuries) reinforce that **component-level failures** can trigger cross-border actions and heightened inspection/handling requirements ([Swissmedic FSCA portal](#); [Swissmedic MAJ-210 FR-1](#)). Expect March–May 2026 to bring continued **FSN volume**, faster escalation from complaint trending to field actions, and more scrutiny of **patient matching / record integrity** in clinical software (e.g., misassignment of ECG reports) ([BfArM – Spacelabs SENTINEL](#)).

- EU/FR: ANSM suspension + withdrawal requirements for a glucose sensor; clinical investigation terminated ([ANSM](#))
- EU/DE: Insulin pump updated warnings to prevent over/under-delivery tied to placement relative to infusion site ([BfArM – Medtronic pumps](#))
- EU/DE/AT/CH: MAJ-210 valve fragment detachment—inspection/handling instructions due to serious injuries ([Swissmedic MAJ-210 FR-1](#))
- Multi-jurisdiction: PACS/reporting workflow defect with correction planned for **Q2 2026**—near-term mitigations required ([BfArM – DeepUnity PACSonWEB](#))

What to Prep Now

- Stand up a **72-hour FSN/recall execution playbook** (roles, customer notification templates, distributor instructions, return logistics, and effectiveness checks) aligned to the most common failure modes (software misassignment, mechanical malfunction, labeling/use-conditions). Evidence basis: repeated FSNs across EU/UK/CH ([BfArM – DeepUnity PACSonWEB](#); [MHRA FSNs](#); [Swissmedic FSCA portal](#)).
 - Implement a **patient-identity and report-linkage control** for clinical software (forced verification steps, audit logs, and regression tests for demographic auto-population and order-to-report linking). Evidence basis: misassignment risks described for ECG reporting and PACS workflow ([BfArM – Spacelabs SENTINEL](#); [BfArM – DeepUnity PACSonWEB](#)).
 - For EU market access: complete **UDI/master data remediation** and map device portfolios to EUDAMED data fields before May 28, 2026 (see Theme 2). Evidence basis: mandatory EUDAMED modules timeline ([EC EUDAMED overview](#)).
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2) EU Device Compliance: EUDAMED Becomes “Mandatory-Use” Infrastructure

A major operational shift lands inside the horizon: the European Commission confirms that, **as of 28 May 2026**, the first four EUDAMED modules are mandatory to use—Actor registration, UDI/Device registration, Notified Bodies & Certificates, and Market Surveillance ([EC EUDAMED overview](#); [EC Getting ready](#)). This is not a policy signal; it is a **hard compliance milestone** that will expose weak data governance, unclear economic-operator roles, and incomplete UDI/device datasets. National authorities are already amplifying the message (e.g., Austria’s BASG summarizing the decision and six-month transition period) ([BASG](#)).

For manufacturers and authorized representatives, the practical implication is that EUDAMED readiness becomes a gating factor for **ongoing EU commercialization** and regulatory interactions. Organizations with fragmented product master data, incomplete certificate mapping, or unclear actor-role assignments should assume higher risk of **registration delays, certificate linkage issues, and market surveillance friction** once mandatory use begins ([EC Actor registration module](#)).

- EU: Mandatory use of first four EUDAMED modules from **28 May 2026** ([EC EUDAMED overview](#))
- EU: Actor module requires economic operators to obtain/maintain SRN and submit required actor information ([EC Actor registration module](#))

- EU/AT: National competent authority communication reinforcing the May 2026 mandatory-use date ([BASG](#))

What to Prep Now

- By **April 15, 2026**, finalize your **economic operator role map** (manufacturer/AR/importer) and ensure SRNs are issued/validated for each legal entity that will transact in EUDAMED. Evidence basis: Actor module purpose and SRN process ([EC Actor registration module](#)).
- Build an **EUDAMED submission “golden dataset”** (UDI-DI, device identifiers, certificate references, NB details) and run a dry-run reconciliation against internal ERP/PLM/quality systems by **May 1, 2026**. Evidence basis: mandatory UDI/Device + NB/Certificates modules ([EC Getting ready](#)).
- Establish a **post–May 28 change-control**: any labeling, IFU, certificate, or UDI change must trigger an EUDAMED update ticket with SLA and owner. Evidence basis: mandatory-use environment and market surveillance module expectations ([EC EUDAMED overview](#)).

3) Pharmacovigilance Tightening: Withdrawals, Labeling Updates, and Counterfeit Signals

Regulators are moving quickly from signal to action in medicines safety. EMA's PRAC recommended withdrawal of levamisole medicines across the EU due to leucoencephalopathy risk and concluded risk mitigation measures were insufficient—an example of **benefit-risk reassessment leading to market exit** ([EMA PRAC highlights](#)). In the UK, MHRA is updating semaglutide product information to include warnings about NAION and directs urgent ophthalmology referral for sudden vision loss—raising immediate obligations for prescribers, pharmacovigilance teams, and patient communications ([MHRA semaglutide NAION](#)).

The UK also highlights falsified GLP-1 products (e.g., falsified Mounjaro pens) as part of its safety roundup, reinforcing that **supply integrity** is now a frontline safety issue, not just a commercial risk ([MHRA Safety Roundup Feb 2026](#)). Expect March–May 2026 to bring more label changes, DHPCs, and heightened scrutiny of distribution channels for high-demand therapies.

- EU: PRAC recommends EU-wide withdrawal of levamisole medicines ([EMA PRAC highlights](#))
- UK: Semaglutide labeling update for NAION risk; urgent referral guidance ([MHRA semaglutide NAION](#))
- UK: Alerts on falsified GLP-1 products and strengthened warnings on GLP-1 class risks ([MHRA Safety Roundup Feb 2026](#))

What to Prep Now

- Update your **signal management SOP** to include a “rapid label change” pathway (medical review, artwork/packaging coordination, HCP comms) with a target cycle time <30 days for high-severity signals. Evidence basis: MHRA NAION update and PRAC withdrawal action ([MHRA semaglutide NAION](#); [EMA PRAC highlights](#)).
 - Implement **anti-counterfeit controls** for high-demand products: distributor due diligence, anomaly detection for returns, and escalation triggers for suspected falsification. Evidence basis: MHRA falsified product alerts ([MHRA Safety Roundup Feb 2026](#)).
 - For EU portfolios: run a **levamisole exposure check** (legacy SKUs, parallel trade, compassionate use) and prepare withdrawal/stock disposition steps where applicable. Evidence basis: PRAC withdrawal recommendation and DHPC dissemination plan ([EMA PRAC highlights](#)).
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4) Supply Continuity & Shortage Management Becomes a Compliance Discipline

Shortages are being managed through formal regulatory communications and exceptional measures, requiring providers and manufacturers to operationalize substitution and conservation plans. EMA reports shortages of ifosfamide and cyclophosphamide expected to last until **Q1 2027** due to manufacturing pauses tied to technical problems and site improvements after inspection—linking **GMP/regulatory inspection outcomes directly to supply continuity** ([EMA Ifosfamide shortage](#); [EMA Cyclophosphamide shortage](#)). Germany’s Paul-Ehrlich-Institut allowed an exception to market an influenza vaccine with Italian labeling for a defined period (Feb 15–Apr 30, 2026), illustrating pragmatic flexibility to address seasonal supply constraints ([PEI Influvac labeling exception](#)).

Sweden’s notice that Natpar will be deregistered and cease delivery by **July 1, 2026** requires proactive patient transitions and prescribing restrictions for new patients ([Swedish MPA Natpar](#)). Australia’s TGA used a Serious Scarcity Substitution Instrument to allow pharmacist substitution of nicorandil strengths/brands through **March 31, 2026**, showing how regulators are expanding pharmacist flexibility under shortage conditions ([TGA nicorandil shortage](#)).

- EU/EEA: Ifosfamide shortage expected until Q1 2027 ([EMA Ifosfamide shortage](#))
- EU/EEA: Cyclophosphamide shortage expected until Q1 2027 ([EMA Cyclophosphamide shortage](#))

- DE: Temporary labeling exception to support influenza vaccine availability (Feb 15–Apr 30, 2026) ([PEI Influvac labeling exception](#))
- SE: Natpar delivery stop and deregistration by July 1, 2026; restrictions for new patients ([Swedish MPA Natpar](#))
- AU: Pharmacist substitution instrument for nicorandil through March 31, 2026 ([TGA nicorandil shortage](#))

What to Prep Now

- Create a **shortage command center** for oncology/critical meds with: (a) SKU-level days-on-hand, (b) therapeutic alternatives, (c) prescriber communications, and (d) patient transition scripts. Evidence basis: EU shortages with long duration to Q1 2027 ([EMA Ifosfamide shortage](#); [EMA Cyclophosphamide shortage](#)).
- For markets allowing exceptions/substitution, publish a **jurisdiction-specific dispensing playbook** (e.g., AU SSSI rules; DE labeling exceptions) and train pharmacy operations by **March 31, 2026**. Evidence basis: AU SSSI and DE exception window ([TGA nicorandil shortage](#); [PEI Influvac labeling exception](#)).
- For product discontinuations (e.g., Natpar), implement a **patient migration tracker** (patient list, alternative regimen, completion date) with a hard stop aligned to expiry/deregistration timelines. Evidence basis: Natpar cessation timeline and patient instructions ([Swedish MPA Natpar](#)).

5) Mandatory Safety Reporting & Quality Governance: From Voluntary to Enforceable

Australia is shifting device vigilance by making healthcare facilities mandatory reporters of medical device adverse events starting **21 March 2026**, with CEO-level accountability and potential civil penalties for non-compliance ([TGA facility reporting guidance](#)). This creates a new data stream (ASDER) designed for trending and signal detection, complementing sponsor/manufacturer reporting pathways ([TGA facility reporting guidance](#)). The policy intent is explicit: faster signal detection and stronger patient safety oversight ([TGA media release 30 Jun 2025](#)).

In France, HAS is tightening patient safety governance through guidance discouraging medication order transcription (a known error vector) and by introducing a labeling process for externally produced patient safety “flash” reports to ensure methodological and ethical standards ([HAS transcription flash](#); [HAS labeling process](#)). Together, these moves indicate regulators are

pushing healthcare delivery organizations toward **standardized, auditable safety processes**—not just clinical best practices.

- AU: Mandatory hospital device adverse-event reporting begins **21 March 2026** ([TGA facility reporting guidance](#))
- AU: Policy rationale—strengthen device patient safety and signal detection ([TGA media release 30 Jun 2025](#))
- FR: Patient safety guidance to reduce prescription transcription errors ([HAS transcription flash](#))
- FR: HAS labeling process for external patient safety flash publications ([HAS labeling process](#))

What to Prep Now

- For AU hospital operators: complete **ASDER onboarding** and define internal triggers so reportable events are captured and submitted within required timelines (e.g., 10-day reporting for death/serious deterioration in Stage 1). Evidence basis: ASDER process and timing requirements ([TGA facility reporting guidance](#)).
- For multinational device manufacturers: update complaint intake to **accept facility-originated ASDER-driven escalations** and reconcile with sponsor reporting to avoid missed signals. Evidence basis: ASDER as complementary signal stream ([TGA facility reporting guidance](#)).
- For FR providers: implement a **no-transcription control** in medication workflows (EHR order entry, read-back, traceability) and audit compliance during care transitions. Evidence basis: HAS transcription risk guidance ([HAS transcription flash](#)).

6) US Payer Controls, Interoperability & State-Level Utilization Management

CMS' Interoperability and Prior Authorization Final Rule (CMS-0057-F) is driving operational requirements beginning **January 1, 2026**, including prior authorization decision timeframes (72 hours urgent; 7 days standard), denial reason specificity, and public reporting of prior authorization metrics (initial set due **March 31, 2026**) ([CMS fact sheet](#)). States are aligning operations accordingly; Colorado's Medicaid program bulletin explicitly references implementing CMS interoperability final rule requirements and reducing prior authorization-related timeframes, alongside billing and reimbursement changes ([Colorado HCPF bulletin B2600535](#)).

Pennsylvania is tightening prior authorization requirements for specific device/prosthetic categories (breast/nipple prostheses; tracheostomy speaking valves/voice prosthetics), reinforcing a broader trend of **granular utilization management** at the state Medicaid level ([PA DHS breast/nipple prostheses PA guidelines](#); [PA DHS tracheostomy/voice devices PA guidelines](#)). For providers and suppliers, March–May 2026 is a period where **denials, documentation demands, and turnaround expectations** will be actively measured and publicly visible.

- US (federal): Prior auth process requirements in effect; public metrics due March 31, 2026 ([CMS fact sheet](#))
- US-CO: Medicaid operational changes tied to CMS interoperability final rule; PA timeframes and billing updates ([Colorado HCPF bulletin B2600535](#))
- US-PA: New prior authorization guidelines for prostheses and speaking valves/voice prosthetics ([PA DHS breast/nipple prostheses PA guidelines](#); [PA DHS tracheostomy/voice devices PA guidelines](#))

What to Prep Now

- For payers: by **March 31, 2026**, publish required prior auth metrics and validate that denial notices include **specific reasons** (template + QA sampling). Evidence basis: CMS metrics and denial requirements ([CMS fact sheet](#)).
- For providers/DME suppliers: build **PA documentation bundles** per PA Medicaid handbook guidance (diagnosis, medical necessity narrative, supporting records) and integrate into EHR/order workflows to reduce rework. Evidence basis: PA prior authorization guideline publications ([PA DHS breast/nipple prostheses PA guidelines](#); [PA DHS tracheostomy/voice devices PA guidelines](#)).
- For Medicaid-facing organizations in CO: adjust revenue cycle ops for **shorter PA pend times** and updated claim modifiers/reimbursement rules referenced by HCPF. Evidence basis: Colorado bulletin operational changes ([Colorado HCPF bulletin B2600535](#)).

7) Enforcement & Accountability: Fraud, Discrimination, Competition, and Transparency

Enforcement remains active and multi-vector. California announced arrests tied to alleged hospice fraud involving enrolling non-terminal patients and fraudulent billing to Medi-Cal and Medicare—continuing a pattern of targeted actions in high-risk billing domains ([CA AG hospice fraud arrests](#)). Massachusetts secured guilty verdicts for unlicensed dentistry, illegal prescribing, and Medicaid fraud—reinforcing that licensure and prescribing controls remain enforcement

priorities ([MA AG Medicaid fraud/unlicensed dentistry][25]). Separately, the EEOC settlement with Geisinger entities (payment plus policy changes, training, and reporting) highlights ongoing exposure for healthcare employers under ADA disability accommodation and retaliation frameworks ([EEOC Geisinger settlement](#)).

On market conduct, the FTC consent order involving Caremark/ACO-related respondents signals continued federal scrutiny of competition issues in healthcare services markets ([FTC consent order PDF](#)). Meanwhile, Colorado's expanded hospital financial transparency reporting (HB23-1226) increases visibility into compensation, cash flows, and capital investment—raising reputational and policy risk for systems with weak community benefit narratives or stressed margins ([Colorado HCPF hospital transparency release](#)).

- US-CA: Hospice fraud arrests tied to Medi-Cal/Medicare billing ([CA AG hospice fraud arrests](#))
- US-MA: Guilty verdicts for unlicensed practice, illegal prescribing, and Medicaid fraud ([MA AG Medicaid fraud/unlicensed dentistry][25])
- US-PA: EEOC disability discrimination/retaliation settlement with healthcare entities ([EEOC Geisinger settlement](#))
- US (federal): FTC consent order addressing Section 5 competition concerns ([FTC consent order PDF](#))
- US-CO: Expanded hospital financial transparency reporting requirements and disclosures ([Colorado HCPF hospital transparency release](#))

What to Prep Now

- Run a **90-day billing integrity sprint** for hospice, DME, and high-risk service lines: eligibility substantiation, physician certification controls, and audit trails for medical necessity. Evidence basis: CA and MA enforcement actions ([CA AG hospice fraud arrests](#); [MA AG Medicaid fraud/unlicensed dentistry][25]).
- For healthcare employers: implement an **ADA accommodation case-management workflow** with documented interactive process steps and anti-retaliation controls; schedule manager training and quarterly reporting. Evidence basis: EEOC settlement terms requiring policy changes/training/reporting ([EEOC Geisinger settlement](#)).
- For hospital systems in transparency states: prepare a **board-ready narrative pack** linking compensation, capital investments, and community benefit to access/quality outcomes before annual reporting cycles. Evidence basis: CO expanded reporting scope ([Colorado HCPF hospital transparency release](#)).